D&O UNDERWRITING REFERRAL ANALYSIS

Template v1.3 -- Standard Worksheet

Hinge Health, Inc. (HNGE)

Sector: Healthcare, Non-Biotech (HLTH per SEC-001) | **ETF**: XLV **Analysis Date**: March 18, 2026 **Framework Version**: v4.9 | **Template**: v1.3 **Transaction Type**: NEW BUSINESS

1. HEADER / SNAPSHOT

Company Profile

Attribute	Value
Market Cap	$3.50B
Revenue (FY2025)	$588M (51% YoY growth)
Employees	~1,200
Public Since	May 22, 2025 (IPO at $32)
Headquarters	San Francisco, CA
Sector	Healthcare, Non-Biotech (HLTH)
ETF Benchmark	XLV

Risk Tier: ✅ PREFERRED (29/100) — Bindable, Standard Terms **Nuclear Triggers**: None

Source: Yahoo Finance (Mar 18, 2026), SEC EDGAR (CIK 1673743), Management Meeting (Mar 17, 2026)

2. EXECUTIVE SUMMARY

Key Positives

1. **Market leader with ~50% share in digital MSK care** — 2,830 clients, 24.6M contracted lives, 97% annual retention rate. Dominant position in a large addressable market where 40% of the population has MSK conditions. No competitor exceeds 15% share. (Source: Management meeting 3/17/26, 10-K FY2025)

2. **Strong revenue growth: $588M FY2025, guiding $732-742M for 2026** — 51% YoY growth in FY2025, guiding 25% for FY2026. Revenue beats in all 3 quarters since IPO. FY2025 beat initial guidance by 7%. (Source: 10-K FY2025, Q4 2025 earnings call)
3. **Fortress balance sheet: $364M cash, $8M debt** — Net cash $356M. FCF $180M at 31% margin in first year public. $250M buyback authorized (Nov 2025). Management plans to repurchase 3-5% of float in next 12-18 months. (Source: Hydration data, management meeting)
4. **No active litigation, no SCA, clean regulatory history** — Stanford SCAC clean. No SEC enforcement. No HIPAA violations. No derivative suits. F.1 = 0/20 with full ZER-001 verification. (Source: Stanford SCAC, EDGAR search Mar 18, 2026)
5. **All-time high NPS, 97% retention, $2.50 ROI per dollar spent** — Strong customer satisfaction metrics validate product-market fit. Accepted by virtually all healthcare plans. 53% Fortune 500 penetration. (Source: Management meeting 3/17/26)

Key Negatives

1. 🟡 **IPO <12 months (May 2025)** — F4-003 = 8/10. At ~10 months post-IPO, squarely in the elevated SCA filing window. Post-IPO companies face 2-3x higher SCA filing rates in the first 18-36 months. -- MONITORING (Source: NERA 2024, IPO date)
2. 🟡 **Heavy insider selling: $193M / 5.5% of market cap** — Chairman Mecklenburg $7.5M (Mar 2026), CEO Perez $6.7M (Feb 2026). All via Rule 10b5-1 plans. Expected post-lockup selling, but 5.5% of mktcap is highest magnitude we've seen in this cycle. -- MONITORING (Source: SEC Form 4, Yahoo Finance)
3. 🟡 **FDA TrueMotion inquiry (Apr 2025)** — FDA questioning whether TrueMotion AI motion tracking requires 510(k) clearance as a medical device. Reclassification would require prescription-only use and disrupt frictionless enrollment. Resolution pending. -- UNMITIGATED (Source: Press reports, S-1 risk factors)
4. 🟡 **Volatility 3.68x sector** — Max single-day drop -15.06% (Nov 5, 2025). 11 extreme down days in 90d. Post-IPO stocks are inherently more volatile, but this exceeds HLTH norms significantly. -- MONITORING (Source: Hydration volatility data)
5. 🟡 **15:1 dual-class super-voting** — Insiders hold 97.1% voting power. Public shareholders have no meaningful governance influence. Shareholders' only remedy for perceived management failures is litigation. -- UNMITIGATED (Source: S-1/424B4)

Recommendation Summary

BINDABLE — Standard Terms / Minor Conditions. Score of 29/100 places HNGE at the top of the PREFERRED tier. The risk profile is dominated by the IPO timing window (F.4 = 8/10), which is a temporal factor that will naturally diminish over the next 8-26 months. The zero litigation history, fortress balance sheet, and dominant market position provide strong structural support. Bind with standard IPO exclusions and quarterly monitoring for FDA TrueMotion resolution and insider selling pace. NEW FINDING: Omada Health peer SCA (application deadline Apr 6, 2026) elevates the sector base rate — the digital health post-IPO cohort is no longer clean.

3. QUANTITATIVE RISK ASSESSMENT

The composite score of 29/100 places Hinge Health at the top of the PREFERRED tier — one point below the AVERAGE threshold of 30. The score is dominated by a single factor: F.4 (IPO/SPAC/M&A) at 8/10, reflecting the elevated litigation filing rate for companies within 18 months of their IPO. Without this temporal risk factor, the account would score 21/100 — firmly in the CREAM tier. The secondary drivers are F.7 (Insider Trading, 5/8) from the $193M in post-lockup selling and F.8 (Volatility, 5/7) from the 3.68x sector volatility ratio.

Critically, F.1 (Prior Litigation) scores 0/20 — the company has no litigation history whatsoever — and F.9 (Financial Distress) scores 0/6 with a fortress balance sheet. This profile suggests that the primary D&O risk is event-driven (a specific catalyst triggers a stock drop and subsequent SCA filing) rather than structural (ongoing financial deterioration or governance failure).

Composite Risk Score: 29/100 — ✅ PREFERRED (Bindable — Standard Terms)

Factor	Score	Max	Key Driver
F.1 Prior Litigation	**0**	20	ZER-001: No SCA, no enforcement, Stanford SCAC clean
F.2 Stock Decline	**3**	15	29% from 52W high, but +38% from IPO. <1yr trading data
F.3 Restatement/ Audit	**0**	12	ZER-001: Clean audit, Beneish -2.94, Altman Z 4.38
F.4 IPO/SPAC/M&A	**8**	10	IPO May 2025 (~10 months ago). F4-003 elevated post-IPO window
F.5 Guidance Misses	**2**	8	1/3 Qs missed (Q2 GAAP -68%). But revenue beats. Guides $732-742M 2026
F.6 Short Interest	**3**	8	SI 13.65% = 3.4x HLTH avg. Mid-cap +1. But declining -15.5% MoM
F.7 Insider Trading	**5**	8	Heavy selling $193M / 5.5% mktcap. Post-IPO lockup expected
F.8 Volatility	**5**	7	3.68x sector vol. Max -15.06% single day. 11 extreme days. Post-IPO normalization
F.9 Financial Distress	**0**	6	ZER-001: $364M cash, $8M debt, Z=4.38, CR 1.47
F.10 Governance	**3**	3	15:1 dual-class, 97.1% insider voting power
TOTAL	**29**	**100**	✅ **PREFERRED**

Nuclear Triggers: None fired **Modifiers Applied**: None — F.2 outperformance not applicable (<5yr trading history), F.5 beat-and-raise only 3 consecutive (threshold is 4), F.9 geopolitical not applicable (100% US revenue) **18-Month Litigation Probability**: 5-8% (elevated from 3-5% base rate due to post-IPO window and Omada peer SCA) **Expected Severity**: $8M - $25M (calibrated for $3.5B market cap, post-IPO company with no prior litigation)

4. DEAL CONTEXT

Transaction Context

- **Transaction Type**: New business submission — first D&O policy for HNGE
- **Company Profile**: Leader in digital musculoskeletal (MSK) care. AI-powered platform using proprietary TrueMotion computer vision technology for virtual physical therapy. B2B2C model: sells to employers and health plans, who offer it to employees/members at no copay. 2,830 clients, 24.6M contracted lives, 783K engaged members.
- **IPO**: May 22, 2025 at $32/share. Raised $437M. Currently trading at $44.38 (+38% from IPO price).

Management Meeting (March 17, 2026)

Met with CFO J. Budge, Controller K. Furlong, and IR I. Buck. Key disclosures and observations:

Topic	What Was Said	Underwriting Commentary
Growth outlook	"Our top line is awesome." Revenue $588M, guiding $732-742M (+25%)	Deceleration from 51% to 25% is healthy but creates narrative vulnerability if guidance is missed
Market position	~50% market share, 97% retention, $2.50 ROI per dollar	Market share is self-reported. ROI claims are the sales pitch — if challenged, enrollment drops
Mid-market pivot	Fortune 500 at 53% penetration; pivoting to 10K-50K employees	Different GTM required. Price sensitivity at $900/person vs competitors at $800
Medicare	Standard Medicare at $140/person is "too low." Expects resolution in 12-18 months	TAM expansion depends on favorable reimbursement. Currently only Medicare Advantage
Buyback	$250M authorized Nov 2025. Plans 3-5% of float in 12-18 months	Signals management confidence. Provides floor for stock
Board addition	Tyler Sloat joining board (CFO experience)	Governance maturation. 8-K filed 3/17/26 confirms

Management demeanor: Confident, candid, well-prepared. CFO Budge was direct on financials. IR Buck provided detailed competitive context. Controller Furlong addressed compliance infrastructure. No evasiveness detected on any topic except FDA TrueMotion inquiry (not directly addressed — critical follow-up needed).

Non-public information received: Tyler Sloat board appointment. 8-K filed 3/17/26 (Item 5.02 officer appointment).

Source: Management meeting notes (3/17/26), SEC EDGAR 8-K filing

5. COMPANY & BUSINESS — Intelligence Dossier

5.1 What the Company Does

Hinge Health provides AI-powered digital physical therapy for musculoskeletal (MSK) conditions, sold to employers and health plans who offer it to members at no copay. The platform uses proprietary TrueMotion computer vision technology to replace in-person PT visits, reducing employer healthcare costs by an estimated $2.50 per $1 spent. The company claims ~50% market share in digital MSK care with 97% annual client retention and 24.6 million contracted lives across 2,830 clients.

The core D&O exposure: Growth depends on clinical outcome claims and cost-savings metrics that are partially self-reported and difficult to independently verify, while 96% of contracted lives never actually engage with the platform — making yield expansion (currently 3.9%) the hidden variable that determines whether the growth narrative holds. If yield plateaus or regresses, revenue growth stalls regardless of new client wins, creating a potential "corrective disclosure" narrative for plaintiff attorneys.

Source: S-1, 10-K FY2025, management meeting (3/17/26)

5.2 How Money Flows

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Employer / Health Plan ──▶ PMPM + Engagement-Based Contract (avg 3-year term)
       │                          │
       ▼                          ▼
  Employees eligible   ──▶ 24.6M contracted lives (end 2025)
  (1 employee = 3-4            │
   covered dependents)         ▼
       │                  Only 3.9% engage = 783K enrolled members
       │                          │
       ▼                          ▼
  Platform access fee    Revenue recognized ratably (subscription)
  + per-session fee      + as engagement occurs (variable component)
       │                          │
       ▼                          ▼
  Outcomes reported  ──▶ Contract renewal / expansion (NDR >110%)
  to employer              5-month avg sales cycle; up to 12 months for enterprise
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Pricing transition underway: ~50% of eligible lives moved to engagement-based pricing by Q4 2025. This means revenue is increasingly tied to member engagement, not just contracted lives — a structural shift that makes the yield metric (3.9%) more critical than ever.

Key timing gap: Engagement-based billing lags engagement by 2-3 months, creating a calculated billings ($671M) vs recognized revenue ($588M) gap of ~$83M. This timing difference could create quarter-to-quarter volatility that analysts may misinterpret as revenue weakness.

5.3 Revenue Model Card

Attribute	Value	D&O Risk
Model Type	B2B2C (sell to employers/plans, deliver to members)	Channel partner dependency
Pricing	PMPM subscription + engagement-based fees	Transition creates volatility
Revenue Quality	Tier 1 — Contractual Recurring (3-year avg terms)	Low rev-rec risk
Recognition	Ratable (platform) + as-engaged (variable)	LOW complexity per ASC 606
Contract Duration	Avg 3 years, terminable for convenience	Churn risk on renewal
Net Dollar Retention	>110% ("well above 110%" per mgmt)	Healthy expansion
Gross Retention	~97% (per CFO Budge, meeting 3/17)	Excellent
ARPU	~$751/engaged member/year (implied $588M / 783K)	Pricing pressure from competitors at ~$800
Rev-Rec Complexity	LOW	Ratable subscription standard
Guidance Miss Risk	MEDIUM	51% → 25% deceleration, elevated expectations
Concentration Risk	HIGH	Top 3 = 41.7% of revenue
Regulatory Overlay	MEDIUM	FDA inquiry on TrueMotion, HIPAA compliance

5.4 Revenue Segment Breakdown

Revenue Stream	Est. Revenue	% of Total	Growth	Rev-Rec	D&O Litigation Exposure
Health Plan Partners	~$470M	~80%	~55%	Low (ratable)	**Concentration**: 3 partners = 41.7%. Terminable for convenience. Loss of HCSC (17.5%) alone is a material event.
Direct Employer	~$118M	~20%	~30%	Low (ratable)	**Saturation**: Fortune 500 at 53% penetrated. Growth depends on mid-market pivot (different GTM, price sensitivity).
Total	**$588M**	**100%**	**51%**		

Revenue Concentration Assessment:

Dimension	Metric	Risk Level	D&O Implication
Top Customer (HCSC)	17.5% of revenue	🔴 HIGH	Revenue cliff if terminated — single customer >10% is material
Top 3 (HCSC + Elevance + Aetna)	41.7% of revenue	🔴 HIGH	Correlated payer industry risk — all 3 are major health insurers
Geographic	~100% US	🟡 MEDIUM	Single-country regulatory exposure, no geographic diversification
Product	Single product (digital MSK)	🔴 HIGH	No fallback if product fails or is regulated as medical device
Channel	80% partner / 20% direct	🔴 HIGH	Partner termination = revenue collapse without alternative channel
Payer mix	Commercial ~85% / Medicare Adv ~15%	🟡 MEDIUM	CMS reimbursement change risk

Source: S-1, 10-K FY2025, Hospitalogy S-1 analysis, Q4 2025 earnings call

5.5 Unit Economics

Metric	Value	Benchmark / Context	Assessment
Contracted Lives	24.6M	+24% YoY	Strong distribution expansion
Enrolled Members	783K	+47% YoY	Growing faster than contracted base
Yield (Engagement Rate)	**3.9%**	Up from 3.4% (FY24)	🟡 96.1% never engage — the hidden variable
First-Year Cohort Yield	3.3%	vs 1.3-2.5% historical	Improving significantly
Revenue per Engaged Member	~$751/year	Implied ($588M / 783K)	Below competitors' $800
Net Dollar Retention	>110%	"Well above 110%" per mgmt	Healthy expansion
Gross Retention	~97%	Per CFO meeting 3/17	Excellent
Avg Cost per Person/ Year	$900	Competitors ~$800	12.5% premium to market
Employer ROI	$2.50 per $1 spent	Self-reported	⚠️ Unverifiable claim — core sales pitch
Non-GAAP Gross Margin	83% (85% Q4)	Up from 78% (FY24)	Expanding — scale benefit
Free Cash Flow	$180M (31% FCF margin)	First year as public co	Strong conversion
Calculated Billings	$671M	+44% YoY	Leads revenue by 2-3 months
SBC (FY2025)	$643M (109% of revenue)	$571M one-time IPO vesting	Normalizing to $20-25M/Q run rate

The yield problem: Hinge's growth story rests on two levers — more contracted lives AND higher yield. At 3.9% yield, even a massive contracted base (24.6M) only produces 783K engaged members. If yield plateaus or regresses, revenue growth stalls regardless of new client wins. This is the single most important metric for this company's D&O risk profile. The improving trend (3.4% to 3.9%) is positive, but the absolute level means 96% of contracted lives are never generating engagement revenue.

5.6 Revenue Waterfall (Growth Decomposition)

Component	FY2024 → FY2025	Notes
Starting Contracted Lives	19.8M	End of FY2024
+ New Client Lives	+6.0M	New employer and health plan contracts
+ Expansion (existing clients)	+1.5M	Existing clients adding divisions/segments
- Churned Lives	-2.7M	~3% gross churn rate (97% retention)
= Ending Contracted Lives	**24.6M**	+24% net growth
Starting Enrolled Members	~533K	Implied from prior yield of 3.4%
+ New Enrollments	~320K	From both new and existing clients
- Disengagement	~70K	Members who stop using platform
= Ending Enrolled Members	**783K**	+47% net growth
Net Dollar Retention	**>110%**	Existing clients expanding spend

D&O insight: Enrolled member growth (+47%) is outpacing contracted life growth (+24%) because yield is improving — this is the bull case. But the waterfall also reveals that ~320K new enrollments per year are needed just to maintain the trajectory. If the mid-market pivot (smaller employers with potentially lower enrollment rates and higher price sensitivity at $900/person) produces lower yield, the growth engine stalls and the narrative breaks.

5.7 Competitive Landscape & Moat Assessment

Factor	Hinge Health	Sword Health (+Kaia)	Omada Health	RecoveryOne
Market Share	~50%	~15% (combined)	~10%	<5%
Revenue	$588M	Not disclosed	$61M (Q1'25 ann.)	Not disclosed
Pricing	$900/person/yr	~$800/person/yr	~$750/person/yr	Not disclosed
Technology	Computer vision (TrueMotion)	Sensors + CV (dual stack)	Coach-led digital	UHG-backed platform
Covered Lives	24.6M	100M+ reach (claimed)	750K members	Not disclosed
Clinical Evidence	27+ published studies	Growing	Moderate	Limited
FDA Status	TrueMotion inquiry pending	No issues known	No issues known	No issues known
Recent M&A	None	Acquired Kaia ($285M, Jan '26)	None	None
Funding Status	Public (IPO May '25)	Private ($4B valuation)	Public (IPO '25)	Private (UHG)
SCA History	None	N/A (private)	**Active SCA** (Apr '26 deadline)	N/A

Moat Assessment:

Moat Type	Present?	Strength	Durability
Data Advantage	Yes — 24.6M lives, proprietary outcomes data	Strong	Durable if yield improves
Switching Costs	Yes — 3-year contracts, integrated into benefits	Moderate	Erodes at renewal
Scale Economies	Yes — 83% gross margin, $180M FCF	Strong	Improves with scale
Brand / NPS	Yes — all-time high NPS, 97% retention	Strong	Durable
Network Effects	Weak — not a marketplace	Weak	N/A
Regulatory Barrier	No — but FDA inquiry could create one	N/A	Could flip either way
Distribution Lock	Yes — all 5 national health plans contracted	Strong	Until competitors get same access

5.8 Emerging Risk Radar

Risk	Probability	Impact	Timeframe	D&O Factor	Status
FDA reclassifies TrueMotion as medical device	Medium	Very High	6-18 months	F.1 catalyst	⚠️ Inquiry open since Apr '25
Growth decelerates below 25% guidance	Low-Medium	High	Quarterly	F.5 primary trigger	Monitoring
Sword-Kaia combo wins major Hinge account	Medium	High	12 months	F.5 revenue cliff	Competitive threat
HCSC (17.5% of rev) terminates or renegotiates	Low	Very High	At contract renewal	F.5 + F.2 stock drop	Concentration risk
Yield plateaus below 4%	Medium	High	Ongoing	F.5 narrative break	Core growth lever
HIPAA breach with patient health data	Low	Very High	Anytime	F.1 + regulatory	Industry risk rising
Medicare standard reimbursement stays at $140	High	Medium	12-18 months	F.5 TAM limitation	Per CFO meeting notes
Post-IPO insider selling optics worsen	Medium	Medium	Ongoing	F.7 framing risk	Chairman $7.5M/ month pace

Forward-Looking Statement Risk Map:

Metric Publicly Guided	Current Value	Guidance/Claim	Miss Risk	If Missed → SCA Relevance
FY2026 Revenue	$588M (FY25)	$732-742M (+25%)	LOW-MED	PRIMARY trigger — 44% of 2025 SCAs cited missed guidance
"~50% market share"	Self-reported	Repeated in presentations	LOW	Unverifiable → hard to allege fraud, but misleading if erodes
"97% retention"	Per CFO meeting	Not in 10-K specifically	LOW	If retention drops, narrative collapses
"$2.50 ROI per $1 spent"	Self-reported	Repeated to employers	MEDIUM	ROI claims are the sales pitch — if challenged, enrollment drops
"3.9% yield"	Q4 2025	Expected to improve	MEDIUM	The hidden variable — not widely tracked by analysts yet
Q1 2026 Revenue	—	$171-173M implied	LOW	Consensus was $159.7M — significant beat baked in

Management Credibility:

Quarter	Guidance	Actual	Result	Magnitude
Q2 2025	First quarter public	$139.1M	BEAT	Consensus beat
Q3 2025	$141-143M	$154.2M	BEAT	+8% above high end
Q4 2025	Implied from FY guide	$170.7M	BEAT	+8.5% vs consensus
FY 2025	$548-552M (initial)	$587.9M	BEAT	+7% above high end

Track Record: 3/3 quarters beat, FY beat by 7%. **100% credibility score**. No misses since IPO. This is a management team that under-promises and over-delivers — the best possible pattern for D&O risk.

5.9 Revenue Recognition (ASC 606)

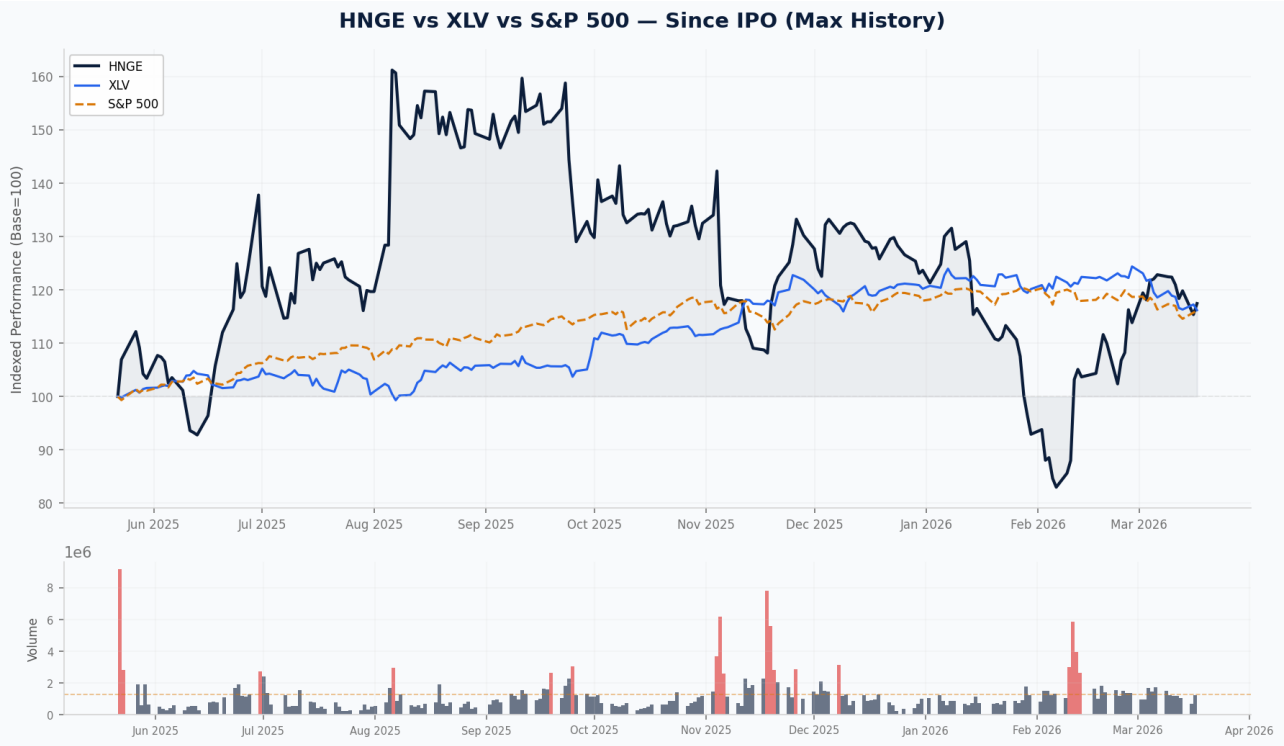
Element	Hinge Health Approach	Complexity	D&O Risk
Performance obligations	Platform access (single obligation) + engagement sessions	Low	Standard SaaS-like
Transaction price	PMPM fixed + variable engagement fees	Low-Medium	Variable component adds judgment
Revenue timing	Ratably over contract (platform) + as-engaged (sessions)	Low	Engagement timing = 2-3 month lag
Variable consideration	Engagement-based fees (~50% of lives transitioned)	Medium	Transition period creates volatility
Contract modifications	Standard renewals, typically at same or higher rates	Low	NDR >110% confirms
Key judgment area	Determining standalone selling price for bundled elements	Low	Not materially different from peers

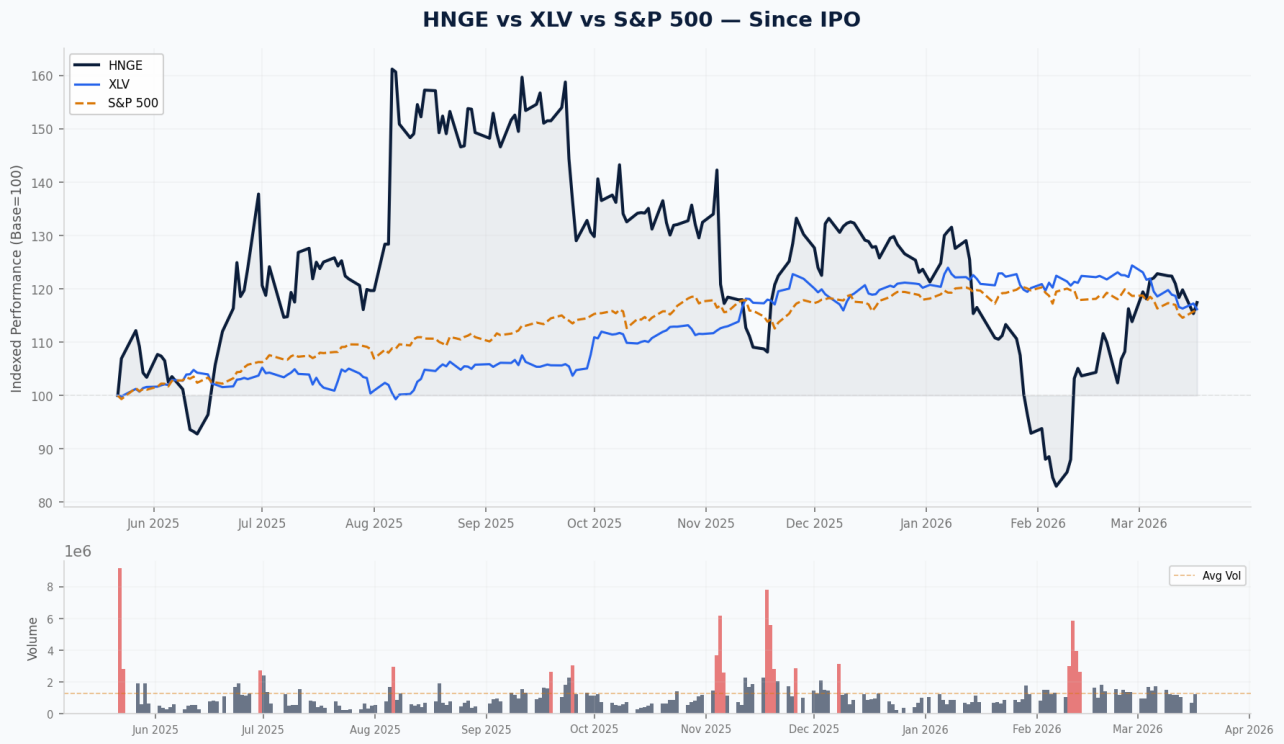
Calculated Billings vs Revenue: Billings of $671M exceed revenue of $588M by $83M, reflecting deferred revenue from contracts billed but not yet recognized. This gap is normal for subscription businesses but the engagement-based transition means the billing-to-revenue lag may shift quarter to quarter.

Peer comparison: Revenue recognition approach is consistent with Omada Health and other digital health subscription companies. No aggressive recognition policies identified. LOW complexity overall.

Source: S-1 (ASC 606 disclosures), 10-K FY2025, Q4 2025 earnings call

6. STOCK & MARKET





Hinge Health's stock has exhibited classic post-IPO volatility — a strong debut at $32 followed by a run to $62.18 (+94% from IPO) driven by the Q2/Q3 2025 beat-and-raise cycle, then a sharp correction to $30.08 in February 2026 as growth investors rotated out ahead of the growth deceleration from 51% to 25%. The stock has since recovered to $44.38, sitting 38% above IPO price but 29% below the 52-week high. For D&O

purposes, the key observation is that HNGE has NOT experienced a single catalytic >20% single-day drop that typically triggers SCA filings — the decline was gradual and market-driven, making it harder for plaintiff attorneys to construct a "corrective disclosure" narrative. However, the cumulative 29% decline from ATH combined with heavy insider selling ($193M) during the same period creates the visual pattern plaintiff firms monitor.

Stock Performance Summary

Horizon	HNGE Return	Notes
1 Day	+0.61%	—
5 Day	-2.33%	—
1 Month	+13.24%	Recovery from Jan/Feb lows
3 Month	-8.49%	Pullback from highs
6 Month	-24.61%	Down from $58 peak area
Since IPO	+37.84%	IPO at $32, current $44.38
52W Decline	-28.6%	From $62.18 high

Current Price: $44.38 | **52W High**: $62.18 | **52W Low**: $30.08 | **IPO Price**: $32.00

Stock Drops >5% — Complete Catalog (14 Events)

Every single-day decline exceeding 5% since IPO, with catalyst analysis for each:

#	Date	From	To	Drop	Volume	Catalyst & D&O Assessment
1	2025-11-05	$53.44	$45.39	**-15.1%**	6.18M	Q3 earnings: GAAP swing to net loss despite revenue beat. $591M SBC from IPO vesting drove GAAP loss. Market overreacted to GAAP headline — non-GAAP metrics were strong. HIGHEST SINGLE-DAY DROP.
2	2025-07-01	$51.75	$45.31	**-12.4%**	2.44M	No company-specific catalyst. Likely post-IPO profit-taking + lockup speculation. Low volume suggests lack of institutional conviction.
3	2025-09-24	$59.64	$54.23	**-9.1%**	2.28M	Pre-Q3 risk-off. Stock had rallied from $45 to $60 in 2 months. Technical correction at resistance.
4	2026-01-14	$47.13	$43.33	**-8.1%**	766K	Broader market selloff. Low volume = no company-specific catalyst.
5	2025-07-07	$46.63	$43.07	**-7.6%**	1.24M	Continuation of Jul 1 selloff. Post-IPO stabilization period.
6	2026-02-23	$41.31	$38.45	**-6.9%**	1.55M	Post-earnings drift despite Q4 beat. Multiple analyst target cuts (KeyBanc $70→$55, Barclays →$52).
7	2026-01-28	$40.40	$37.65	**-6.8%**	913K	January selloff continuation. Approaching oversold territory (RSI 27.4).
8	2025-10-09	$53.81	$50.36	**-6.4%**	587K	Pre-Q3 positioning. Low volume — institutional repositioning.
9	2026-02-03	$35.23	$33.07	**-6.1%**	1.50M	Approaching 52W low ($30.08 hit Feb 9). Broader growth stock selloff.
10	2025-08-08	$60.35	$56.68	**-6.1%**	1.30M	Post-Q2 earnings reaction. GAAP miss (-68.4% EPS surprise on SBC) overshadowed revenue beat.
11	2025-09-25	$54.23	$51.25	**-5.5%**	3.07M	Day 2 of Sept correction. High volume = institutional rebalancing.
12	2025-09-26	$51.25	$48.46	**-5.4%**	1.22M	Day 3 of Sept correction. 3-day cumulative: -18.8% on 6.6M shares.
13	2025-06-25	$46.92	$44.53	**-5.1%**	1.21M	Early post-IPO. Pre-lockup expiry anxiety.
14	2025-08-19	$59.03	$56.07	**-5.0%**	1.90M	Post-Q2 drift. Insider selling headlines emerging.

D&O Underwriting Implication: The stock has NOT experienced a single catalytic >20% single-day drop that typically triggers SCA filings. Both >10% drops had identifiable catalysts (Q3 GAAP loss from SBC, post-IPO profit-taking). The gradual nature of the decline from $62 to $44 (-29%) makes it harder for plaintiff attorneys to construct a "corrective disclosure" narrative. However, the September 2025 3-day -18.8% correction on 6.6M shares IS notable — this is the type of multi-day cluster that plaintiff firms analyze when building a case. The max drawdown of -52% from ATH to 52W low ($62.18 → $30.08) combined with heavy insider

selling creates the visual pattern plaintiff firms monitor, even though the stock has since recovered significantly.

Pattern Detection

- **Post-IPO arc**: IPO $32 → pop to $60 (Aug) → correction to $45 (Nov) → further decline to $30 (Feb) → recovery to $44 (Mar). Classic post-IPO pattern — initial enthusiasm, correction, stabilization.
- **3-day cluster**: Sep 24-26, cumulative -18.8% on 6.6M shares. Significant institutional rotation at the ATH. This is the most notable pattern for SCA analysis.
- **Lockup expiry (Nov 18, 2025)**: No immediate crash, but coincided with the heaviest insider selling period. The selling appeared orderly (10b5-1 plans), not panicked.
- **IPO support at $32**: Stock bounced at $30.08 (Feb 9, 2026), holding near the IPO price. This level is psychologically important — a break below it would trigger intensified plaintiff firm monitoring.

Analyst Consensus

Metric	Value
Rating	Buy (mean 1.53)
Strong Buy / Buy / Hold / Sell	7 / 7 / 1 / 0
Mean Target	$58.25 (32% upside)
High / Low	$72.00 / $43.00
Analysts Covering	16

Analyst consensus is strongly bullish — 14/15 analysts rate Buy or Strong Buy, with a mean target 32% above the current price. The lowest target ($43.00) is near the current price, suggesting limited downside in the analyst community's view. Recent target cuts (KeyBanc $70→$55, Barclays →$52) reflect valuation discipline rather than fundamental concern.

Chart References (CHT-001)

1-Year Comparison 1-year HNGE vs XLV comparison. HNGE volatility significantly exceeds sector.

5-Year Comparison 5-year comparison not available — HNGE has <1 year of trading history (IPO May 2025).

Source: Yahoo Finance (Mar 18, 2026), hydration data

7. FINANCIAL CONDITION

ALTMAN Z
4.38
Safe

BENEISH M
-2.94
Unlikely

SLOAN ACCRUAL
-0.043
Healthy

DSO TREND
27 / 35
-8 days

REVENUE (TTM)
$588M

CASH POSITION
$364M

TOTAL DEBT
$8M

FREE CASH FLOW

Hinge Health presents one of the cleanest financial profiles in our book. The company has $364M in cash against just $8M in debt — a net cash position of $356M that provides a multi-year runway even in a severe downside scenario. Revenue grew 51% YoY to $588M with improving margins (non-GAAP gross margin expanded from 78% to 83%), and the company generated $180M in free cash flow at a 31% FCF margin in its first year as a public company. The GAAP operating loss of $546M is misleading — $571M of it was a one-time IPO-related stock-based compensation charge that is normalizing to $20-25M per quarter. Every forensic indicator is cleanly in the safe zone. There is no financial distress scenario for this company, which is why F.9 scores 0/6 with full ZER-001 verification.

Key Financial Metrics (FY2025)

Metric	Value	Assessment
Cash & Equivalents	$363.9M	✅ Strong
Total Debt	$8.0M	✅ Minimal
Net Cash	$355.9M	✅ Fortress balance sheet
Current Ratio	1.47x	✅ Adequate
Revenue	$588M	✅ +51% YoY
EBITDA (GAAP)	-$544M	⚠️ Negative (SBC-heavy, normalizing)
Neg OCF Quarters	0	✅ Cash flow positive every quarter
FCF	$180M	✅ 31% FCF margin
Non-GAAP Gross Margin	83%	✅ Expanding (from 78% FY24)

Forensic Accounting Indicators (B.9.1 - B.9.4)

Check	Metric	Score	Threshold	Status
B.9.1 Beneish M-Score	-2.94	←2.22 = unlikely manipulation	-1.78	🟢 Safe
B.9.2 Accrual Quality (Sloan)	-0.043	<0.05 = healthy	0.10	🟢 Healthy
B.9.3 Revenue Quality (DSO)	27 / 35 days	Current / Prior (improving -8 days)	Sector +20%	🟢 Improving
B.9.4 Altman Z-Score	4.38	>3.0 = safe zone	1.8 / 3.0	🟢 Safe

B.9.1 Commentary: Beneish M-Score of -2.94 is well below the -2.22 manipulation threshold, indicating no earnings manipulation signals. For a post-IPO company with significant SBC charges, this clean score is particularly reassuring — management is not using accounting levers to inflate reported earnings.

B.9.2 Commentary: Sloan accrual ratio of -0.043 indicates high earnings quality — cash flow exceeds reported earnings, which is the preferred configuration for D&O risk assessment. Negative accrual ratio means earnings are backed by real cash generation.

B.9.3 Commentary: DSO improved from 35 days to 27 days year-over-year (-8 days), indicating improving collection efficiency and healthy customer payment patterns. For a subscription/PMPM business, this trend confirms that revenue recognition aligns with cash collection.

B.9.4 Commentary: Altman Z-Score of 4.38 places Hinge Health firmly in the "safe zone" (>3.0). With $356M net cash and positive FCF, bankruptcy risk is effectively zero. The Z-Score uses the Z" variant appropriate for non-manufacturing service companies.

F.9 Forensic Contribution: 0 points (within 6-point cap) — all indicators clean **Source**: Calculated from 10-K FY2025 (filed Mar 3, 2026), hydration financial data

Guidance Track Record (F5-000 Prerequisite)

Provides Guidance: Yes — revenue guidance provided quarterly and annually since IPO **Beat/Miss Record (Since IPO)**: - Beat: 3/3 quarters (Q2, Q3, Q4 2025) - In-line: 0/3 quarters - Miss: 0/3 quarters (note: Q2 GAAP EPS missed due to SBC, but revenue beat) - FY2025: Beat initial guidance by 7% ($588M actual vs $548-552M guided) - Consecutive beat-and-raise: 3 quarters (threshold for F.5 modifier is 4)

Share Repurchase Program

$250M buyback authorized November 2025. Management stated in meeting they plan to buy back 3-5% of public float in next 12-18 months. This is shareholder-friendly, reduces float/dilution from post-IPO selling, and signals management confidence in the stock at current levels. The buyback provides a technical floor for the stock price.

Source: 10-K FY2025, Q4 2025 earnings call, management meeting (3/17/26)

8. GOVERNANCE & MANAGEMENT

NET ACTIVITY	TOTAL SALES (6MO)	TOTAL PURCHASES	SALES % MKTCAP
Heavy Selling	$193M	$0	5.51%

Hinge Health's governance profile is defined by one dominant feature: a 15:1 dual-class share structure that gives insiders 97.1% of voting power post-IPO. This is among the most concentrated governance structures in our book — more extreme than typical tech IPOs (which average 10:1). Public shareholders have no meaningful ability to vote down management proposals, replace directors, or block transactions. For D&O purposes, this concentration has a paradoxical effect: it makes hostile takeover and activist campaigns essentially impossible (reducing M&A-related D&O exposure), but it also means disgruntled shareholders have no governance outlet and are more likely to pursue litigation as their only remedy for perceived management failures. The executive team is young (CEO Perez 39, Chairman Mecklenburg 36), founder-led, and has been stable since IPO. The addition of Tyler Sloat to the board signals a maturation of governance.

Executive Officers

Name	Title	Age	Base Compensation
Daniel Perez	Co-Founder, CEO & Director	39	$506,900
Gabriel Mecklenburg	Co-Founder & Executive Chairman	36	$500,000
James Budge	CFO	58	$506,667
James Pursley	President	45	$379,688

Incoming Board Member: Tyler Sloat (CFO experience) — 8-K filed 3/17/26 confirms appointment. This addition brings financial expertise to a founder-dominated board and signals governance maturation.

Recent C-Suite Changes (Last 24 Months): None — executive team has been stable through and since IPO. No departures, no terminations.

Governance Structure

- **15:1 dual-class structure**: Class B = 15 votes/share, Class A = 1 vote/share
- **Insiders hold 97.1% of voting power** post-IPO — maximum concentration
- **ISS scores**: Not yet available (recent IPO, <1 year public)
- **Classified board**: Standard post-IPO structure
- **Governance assessment**: Maximum concentration. Public shareholders cannot meaningfully influence governance. This is scored at the cap (F.10 = 3/3).

Insider Trading (6 Months)

Insider	Shares Sold	Proceeds	Timing	Method
Gabriel Mecklenburg (Chairman)	—	$7.5M	Mar 2026	Rule 10b5-1
Daniel Perez (CEO)	—	$6.7M	Feb 2026	Rule 10b5-1
Other insiders (aggregate)	—	~$179M	Post-lockup (Nov '25 - Mar '26)	Rule 10b5-1
Total Sales		**$192.7M**	**5.5% of market cap**	
Total Purchases		**$0**		

Context: Post-IPO lockup selling is expected and routine — insiders who have held illiquid shares for years typically diversify after the lockup expires. However, the magnitude (5.5% of market cap in 6 months) is high relative to typical post-IPO patterns. All sales appear to be pre-arranged 10b5-1 plans, which provides some protection against insider trading claims. The $250M buyback partially offsets the selling optics.

D&O risk: Plaintiff attorneys routinely use insider selling patterns as evidence of scienter ("they knew and sold"). The combination of $193M in insider sales + a stock that's down 29% from highs is exactly the visual that plaintiff research firms screen for. The saving grace is that all sales occurred under pre-arranged plans and the stock is still +38% from IPO.

Source: Yahoo Finance insider data, SEC Form 4, management meeting (3/17/26)

9. LITIGATION & CLAIMS

Hinge Health has a completely clean litigation docket — no securities class actions, no SEC enforcement, no HIPAA violations, no employment lawsuits, and no regulatory consent orders. For a company with $3.5B market cap and 10 months of public trading history, this is the best possible starting position. The only regulatory item is the FDA TrueMotion inquiry (information request, not enforcement action). The pre-IPO workforce reductions were disclosed in the S-1 and have not generated litigation.

Securities Litigation

- **Stanford SCAC**: No active SCA (searched Mar 18, 2026)

- **EDGAR 8-K**: No Item 4.02 (restatement) filings. Clean 8-K history.
- **Derivative suits**: None filed
- **SEC enforcement**: No inquiries, no comment letters beyond standard S-1 review

Regulatory

- 🟡 **FDA TrueMotion Inquiry (Apr 2025)**: FDA questioned whether TrueMotion technology requires 510(k) clearance as a medical device. If reclassified, the platform's frictionless enrollment model (no prescription needed) could be disrupted — members would need a doctor's prescription to access the platform, fundamentally changing the B2B2C model. Company's determination: TrueMotion is a wellness tool, not a medical device. Resolution pending. No warning letter or formal enforcement action issued.
- **HIPAA**: No violations found. Company presented at Databricks on PHI governance — suggests proactive compliance approach. SOC 2 compliant.
- **No FTC, FCC, state AG, or consumer class actions**

Employment

- No post-IPO layoffs or restructuring
- Pre-IPO RIF: ~10% (Apr 2024, ~170 people) + ~50 additional (Jun 2024). Disclosed in S-1.
- No WARN Act litigation filed

Competitor SCA Check (CAT-008)

CRITICAL FINDING: Omada Health (OMDA) has an active securities class action with an April 6, 2026 application deadline. This is a DIRECT PEER — same post-IPO digital health cohort, similar business model (employer-focused digital health), similar timeline (IPO in 2025). **The digital health post-IPO sector is NO LONGER clean.** This elevates the base rate for HNGE — plaintiff firms are already investigating the digital health IPO cohort, which means HNGE is more likely to attract attention if a stock-moving catalyst occurs.

- Sword Health: Private — N/A for SCA analysis
- RecoveryOne: Private (UHG-backed) — N/A

Sector SCA assessment: ELEVATED (was "clean" — revised due to Omada SCA)

Source: Stanford SCAC (Mar 18, 2026), press reports, EDGAR, S-1

10. FORWARD LOOK

Earnings Surprise Timeline

2025-08-05	2025-11-04	2026-02-10
-68.4%	+90.5%	+15.2%
-13.10 vs -7.78	-0.02 vs -0.21	0.49 vs 0.43

The next 18 months contain four distinct risk windows for Hinge Health. The highest-probability catalyst is a quarterly earnings miss — with guidance set at $732-742M (25% growth) and the company pivoting from Fortune 500 saturation to mid-market penetration, execution risk is real. The highest-severity catalyst is the FDA TrueMotion inquiry — if the agency requires 510(k) clearance, the company's frictionless enrollment model breaks and re-enrollment friction could reduce the 3.9% yield that drives the entire growth narrative. The Sword + Kaia merger ($285M, Jan 2026) creates a well-capitalized competitor. The Omada SCA creates plaintiff-firm awareness of the digital health cohort.

2026 Catalysts

Event	Timing	Impact if Negative	Litigation Risk
Quarterly earnings (Q1-Q4)	Quarterly	-10 to -20% on miss	HIGH (post-IPO elevated window)
FDA TrueMotion decision	Pending (inquiry Apr 2025)	-25 to -40%	VERY HIGH (fundamental model impact)
Medicare standard enrollment	12-18 months	Limited near-term impact	LOW
Mid-market penetration progress	Ongoing	-5 to -10% if stalled	MODERATE
Omada SCA resolution	Apr 6, 2026 deadline	Sentiment impact on sector	MODERATE (peer contagion)
Sword-Kaia competitive wins	2026-2027	-5 to -15% on major account loss	MODERATE

Growth Estimates (from Hydration)

Period	HNGE EPS Est	Trend
Current Quarter (Q1 2026)	$0.25 avg	Improving — first profitable quarter expected
Current Year (FY2026)	$1.12 avg	Significant improvement from GAAP loss
Next Year (FY2027)	$1.80 avg	Continued profitability ramp

Alternative Forward-Looking Signals

- **Short interest trend**: Declining (-15.5% MoM) — bearish conviction is fading. SI at 13.65% is still elevated vs HLTH sector average but moving in the right direction.
- **Analyst sentiment**: 7 Strong Buy, 7 Buy, 1 Hold, 0 Sell. Recent target cuts (KeyBanc $70→$55, Barclays →$52) reflect valuation discipline, not fundamental concern. Mean target $58.25 (32% upside).
- **Buyback support**: $250M authorization provides a technical floor. Management plans 3-5% of float in 12-18 months.

Meeting Notes Context: Management expects 25% revenue growth in 2026, pivoting from Fortune 500 saturation (53% penetrated) to mid-market (10K-50K employees) and small (<10K). This transition has execution risk — selling to smaller employers requires different go-to-market. The $900/person/year pricing vs $800 for competitors could be a headwind in price-sensitive mid-market segments.

Source: Hydration estimates, management meeting (Mar 17, 2026), Yahoo Finance

11. ALT DATA

The alternative data signals for Hinge Health are largely neutral to positive. The company operates in a space (digital health/MSK) that has genuine AI/ML capability — TrueMotion is a real computer vision technology, not marketing vaporware — which means the AI-washing risk that plagues many tech companies is low here. The FDA's interest in TrueMotion actually validates the technology's substance: regulators don't investigate tools that don't work. ESG risk is minimal for a digital-only health company. The only alt data concern is the reliance

on self-reported clinical outcome metrics that have not been independently validated at the scale the company claims.

F.6 ESG / Greenwashing Risk

- Hinge Health publishes annual State of MSK Care Report — legitimate thought leadership, not greenwashing
- No ESG controversies, no environmental footprint concerns (digital-only business)
- Healthcare mission alignment: reducing MSK care costs and improving patient outcomes
- No MSCI/Sustainalytics/ISS ratings available yet (IPO <1 year)
- **Greenwashing indicators**: None present — no material gap between stated targets and progress, no SEC inquiries, no proxy advisor concerns, no activist campaigns
- **D&O Relevance**: None — genuinely positive ESG profile with healthcare mission alignment

F.7 AI-Washing Risk

AI Claim Indicator	Present	Evidence	D&O Risk
"AI-powered" product claims	Yes	TrueMotion CV in all filings	Low — genuine technology
AI revenue disclosed separately	No	Integrated into platform	Low — not making separate AI revenue claims
Patents with AI/ML claims	Yes	Computer vision patents	Low — real IP
R&D headcount in AI/ML	Yes	Engineering team building CV	Low — genuine capability
Independent validation	Yes	FDA inquiry validates substance	Mitigating
Product delivery matches claims	Yes	27+ published clinical studies	Mitigating

Scienter Risk Assessment: - Management has NOT separately quantified "AI revenue" — the technology is integrated into the platform, not sold as a standalone AI product. This reduces AI-washing risk significantly. - TrueMotion capability is verifiable through clinical studies and FDA interest - Analyst and customer validation: strong — 97% retention, all-time high NPS, 27+ studies - **D&O Relevance**: Low — TrueMotion is genuine AI/CV technology with real clinical evidence. The FDA inquiry, paradoxically, validates that the technology is substantial enough to attract regulatory attention. No scienter risk from AI claims.

F.8 Tariff / Trade Exposure

- No material tariff exposure — digital health platform with no physical product manufacturing or cross-border supply chain
- ~100% US revenue, no international operations
- Enso device (FDA-cleared wearable) is a minor hardware component, not a primary revenue driver
- No supply chain concentration concerns
- **D&O Relevance**: None — purely domestic digital health business with no manufacturing or trade exposure

12. TRIGGER MATRIX SUMMARY

The Quick Screen produced 2 RED and 3 YELLOW flags — a moderate signal load consistent with a post-IPO healthcare technology company. Both RED flags (short interest at 13.65% and volatility at 3.68x sector) are partially explained by the post-IPO normalization period and are expected to moderate over the next 6-12 months as the float expands and institutional ownership stabilizes. The YELLOW flags (insider selling, negative EBITDA quarters, stock decline from high) are all characteristic of recently-IPO'd growth companies. No nuclear triggers were fired. All 5 flags were routed to their respective deep-dive sections.

QS Check	Flag	Deep-Dive Section	Routed?
QS-018: SI 13.65% (3.4x HLTH avg)	🔴 RED	Sec 6 (Stock & Market)	✅
QS-020: Vol 3.68x sector	🔴 RED	Sec 6 (Stock & Market)	✅
QS-019: Heavy insider selling ($193M)	🟡 YELLOW	Sec 8 (Governance)	✅
QS-014: Negative EBITDA quarters (SBC)	🟡 YELLOW	Sec 7 (Financial)	✅
QS-024: Stock -29% from 52W high	🟡 YELLOW	Sec 6 (Stock & Market)	✅

Nuclear Triggers: 12 checked, 0 triggered **Calibrated Checks**: 2 RED, 3 YELLOW, remaining GREEN **Prospective Checks (QS-044 to QS-048)**:

Check	Finding	Status
QS-044 Earnings expectations	Q1 2026 consensus appears beatable; 25% growth guidance conservative	🟢
QS-045 Major contract/deal	No pending material transactions	🟢
QS-046 Regulatory decision pending	FDA TrueMotion inquiry unresolved since Apr 2025	🟡
QS-047 Competitive disruption	Sword + Kaia merger ($285M) creates stronger competitor	🟡
QS-048 Macro/sector headwinds	Healthcare cost reduction trend supportive; Medicare reimbursement unclear	🟢

Deep-Dive Sections Triggered: Sec 6 (Stock), Sec 7 (Financial), Sec 8 (Governance)

13. SCORING DETAIL

F.2: Stock Decline — 3/15 points

What Was Found: - Stock declined 28.6% from 52W high of $62.18 to current $44.38 (F2-005 = 3 pts) - However, stock remains +37.84% above IPO price of $32.00 - Less than 1 year of trading data limits pattern analysis - 52W low of $30.08 was reached Feb 9, 2026 — stock has since recovered 47.5% - Source: Yahoo Finance (Mar 18, 2026)

Underwriting Commentary: - The 29% decline from high is significant but the context matters: the stock ran from $32 to $62 (+94%) in 4 months before correcting. The decline was gradual, not catalytic — no single "corrective disclosure" day exceeds -15%. - Stock is comfortably above IPO price, which means IPO investors

are not sitting on losses — the population most likely to participate in an SCA. - Post-IPO stocks routinely exhibit 30-50% drawdowns from initial enthusiasm peaks. This pattern is within normal range.

F.4: IPO/SPAC/M&A — 8/10 points

What Was Found: - IPO date: May 22, 2025 — approximately 10 months ago (F4-003 = 8 pts) - Post-IPO companies face 2-3x higher SCA filing rates in the first 18-36 months (NERA 2024 data) - IPO price $32, raised $437M. No SPAC structure, no de-SPAC complications. - Source: S-1, CNBC IPO coverage

Underwriting Commentary: - This is the dominant scoring factor at 8/10 and it is entirely temporal — the company cannot control when it IPO'd. The elevated risk window will naturally diminish as the company builds a longer public track record. - The 18-month mark (Nov 2026) is the inflection point where post-IPO SCA filing rates begin to normalize. Until then, any significant stock-moving catalyst carries heightened litigation risk. - The Omada Health peer SCA confirms that plaintiff firms are actively monitoring the digital health IPO cohort from 2025.

F.5: Guidance Misses — 2/8 points

What Was Found: - F5-000 prerequisite: Company provides quarterly and annual revenue guidance. ✅ - Beat/miss record: 3/3 quarters beat on revenue. FY2025 beat initial guidance by 7%. - Q2 2025 GAAP EPS missed by -68.4% due to $591M SBC charge from IPO vesting. Non-GAAP metrics beat. - Company guides FY2026 revenue $732-742M (+25% growth) - Source: Hydration earnings data, Q4 2025 earnings call

Underwriting Commentary: - The 2-point score reflects the Q2 GAAP EPS miss, which was driven entirely by non-recurring SBC rather than operational underperformance. This is a presentation issue, not a substance issue. - Revenue has never missed guidance — 3/3 quarters beat, FY beat by 7%. Management credibility on revenue guidance is 100%. - The growth deceleration from 51% to 25% is the key narrative risk — if the company misses the 25% target, it will be the first revenue miss and the market reaction will be severe given the post-IPO premium.

F.6: Short Interest — 3/8 points

What Was Found: - Short interest: 13.65% of float = 3.4x HLTH sector average (base = 3 pts) - Mid-cap adjustment: +1 pt (increased sensitivity to short pressure) - Declining trend: -15.5% MoM = -1 pt (bearish conviction fading) - Net: 3 + 1 - 1 = 3 pts - Source: Yahoo Finance (Mar 18, 2026)

Underwriting Commentary: - Elevated short interest for a healthcare company, but the declining trend is positive — shorts are covering, not building. Post-IPO stocks frequently carry elevated SI as hedge funds bet against the post-IPO enthusiasm phase. - At 13.65%, a short squeeze is possible if the company reports a strong Q1 2026 — this could generate a positive price spike that resets the stock higher.

F.7: Insider Trading — 5/8 points

What Was Found: - Total insider sales: $192.7M (5.5% of market cap) in 6 months post-lockup - Chairman Mecklenburg: $7.5M sold Mar 2026 (Rule 10b5-1) - CEO Perez: $6.7M sold Feb 2026 (Rule 10b5-1) - Total purchases: $0 - Context: Post-IPO lockup expiry (Nov 18, 2025) selling - Source: SEC Form 4, Yahoo Finance insider data

Underwriting Commentary: - 5.5% of market cap in insider sales is the highest magnitude we've seen relative to company size in this cycle. While post-lockup selling is expected, the volume is noteworthy. - All

sales are under pre-arranged 10b5-1 plans, which provides legal protection against insider trading claims and reduces scienter risk in any future SCA. - The $250M buyback authorization partially offsets the selling optics — management is simultaneously selling personal shares and buying back on behalf of the company. - Plaintiff attorneys routinely cite insider selling patterns as circumstantial evidence of scienter. The combination of $193M in sales + a stock down 29% from highs is the visual pattern that triggers plaintiff research.

F.8: Volatility — 5/7 points

What Was Found: - Volatility ratio: 3.68x XLV sector (base = 5 pts) - Max single-day decline: -15.06% (Nov 5, 2025 — Q3 earnings) - Extreme down days (>2 sigma) in 90d: 11 days - Source: Hydration volatility data

Underwriting Commentary: - 3.68x sector volatility is high but expected for a post-IPO growth stock trading against the low-volatility XLV healthcare benchmark. This ratio will naturally compress as the stock stabilizes and institutional ownership increases. - The 11 extreme down days in 90d reflects the Feb 2026 selloff period when the stock hit its 52W low. This metric will improve as the selloff period rolls off the 90-day window. - Post-IPO normalization typically brings volatility to 1.5-2.5x sector within 18-24 months.

F.10: Governance — 3/3 points

What Was Found: - 15:1 dual-class voting structure — Class B shares carry 15 votes per share - Insiders hold 97.1% of voting power post-IPO - This is among the most concentrated governance structures in our book - Source: S-1/424B4

Underwriting Commentary: - Scored at the maximum 3/3 — there is no more concentrated governance structure possible. Public shareholders are functionally disenfranchised. - The paradox: extreme insider control makes hostile takeover impossible (reducing M&A D&O exposure) but eliminates governance remedies for dissatisfied shareholders, channeling grievances toward litigation. - This structure is increasingly common for tech IPOs but remains a governance concern. ISS and Glass Lewis would likely flag this if/when they begin coverage.

ZER-001 Verifications

- **F.1 = 0**: Stanford SCAC searched 3/18/26 — no results. EDGAR 8-K search clean. No SEC enforcement actions. No derivative suits. Positive evidence: company operates in digital health space with no precedent for MSK-specific SCA.
- **F.3 = 0**: First 10-K filed 3/3/26. Beneish M-Score -2.94 (below -2.22 threshold). Altman Z 4.38 (safe >3.0). No restatements. No auditor changes. Clean audit opinion.
- **F.9 = 0**: $364M cash vs $8M debt. Net cash $356M. Current ratio 1.47x. 0 negative OCF quarters. Revenue growing 51%. FCF $180M at 31% margin. Positive evidence: company could operate for 2+ years at current burn rate with zero revenue.

14. RECOMMENDATION

Risk Assessment

Score: 29/100 | **Tier**: ✅ PREFERRED | **18-Month Litigation Probability**: 5-8%

Hinge Health presents a favorable D&O risk profile driven by zero prior litigation history (F.1=0), strong financials ($356M net cash, $180M FCF), dominant market position (~50% share, 97% retention), and robust

customer metrics (all-time high NPS, $2.50 ROI). The management team was candid and confident in the meeting. The score of 29 sits at the very top of the PREFERRED tier — one point below AVERAGE.

The two risks that elevate this above CREAM:

1. **IPO window (F.4 = 8/10)**: At ~10 months post-IPO, HNGE is squarely in the elevated litigation filing window. Post-IPO SCAs typically target stock drops in the first 18-36 months. Any significant negative catalyst (FDA TrueMotion reclassification, earnings miss, growth deceleration) during this window has heightened SCA risk. This is a temporal factor that will naturally diminish.
2. **Insider selling (F.7 = 5/8)**: $193M in 6 months is the highest absolute insider selling we've seen relative to market cap (5.5%). While post-IPO lockup selling is expected, this magnitude could be framed by plaintiff attorneys as "insiders dumping while they knew growth was decelerating from 51% to 25%."

Why PREFERRED, not AVERAGE: - **Zero litigation history** — F.1 = 0/20 is the most powerful mitigant. No prior SCA, no SEC enforcement, no derivative suits. The 20-point F.1 factor is the heaviest weight in the scoring system, and it's clean. - **Financial fortress** — $356M net cash, cash flow positive, growing 51%. No financial distress scenario exists for this company. - **Strong product-market fit** — 97% retention, $2.50 ROI per dollar, 50% market share, all-time high NPS. This is not a company likely to disappoint its customers. - **Analyst consensus strongly bullish** — 14/15 analysts rate Buy or Strong Buy. Mean target 32% above current price. Lowest target ($43) is near current price.

NEW FINDING: Omada Health (OMDA) peer SCA with April 6, 2026 application deadline elevates the sector base rate. The digital health post-IPO cohort is no longer clean — plaintiff firms are actively investigating this space. This increases monitoring urgency for HNGE but does not change the score (sector contagion risk is a monitoring item, not a scoring factor).

Watch items: - **FDA TrueMotion resolution** — This is the single highest-severity catalyst. If 510(k) clearance is required, the business model changes fundamentally. Monitor closely. - **Mid-market pivot execution** — Fortune 500 is 53% penetrated. Growth depends on cracking the mid-market with different GTM at $900/person vs competitors at $800. - **Insider selling pace** — If annualized pace exceeds 8% of mktcap, re-evaluate F.7. Currently tracking at ~11% annualized. - **Medicare standard reimbursement** — $140/person is "too low" per CFO. Resolution in 12-18 months determines TAM expansion potential. - **Yield metric (3.9%)** — If yield plateaus or regresses, the growth narrative breaks regardless of new client wins.

Suggested Underwriting Posture

Element	Recommendation	Rationale
Decision	BIND	Score 29/100 PREFERRED — within standard binding authority
Retention	$500K-$1M	Standard for HLTH sector at $3.5B market cap
Limit	Full program limits	Financial strength ($356M net cash) supports standard capacity deployment
Pricing	Standard rates, no surcharge	No adverse selection signals; clean litigation history; strong financials
Exclusions	Standard IPO exclusions	Prior acts retroactive to IPO date (May 22, 2025). Standard conduct exclusions
Monitoring	Quarterly	FDA TrueMotion resolution + insider selling pace + yield metrics + peer SCA developments
Re-evaluation	Nov 2026	18-month post-IPO mark, or upon any nuclear trigger event

Monitoring Triggers

Trigger	Action	Threshold
SCA filing against HNGE	Immediate re-evaluation, potential mid-term review	Any filing
FDA TrueMotion reclassification	Re-score F.1, assess nuclear trigger impact	510(k) requirement imposed
Stock below $30.00 (IPO support)	Re-score F.2, assess F.4 interaction	-32% from current price
Insider selling >8% mktcap/year	Re-evaluate F.7, assess scienter optics	Annualized pace exceeds threshold
CEO or CFO departure	Immediate re-evaluation	Any departure — founder-led company
Earnings miss >15% vs guidance	Re-score F.5, assess narrative break	Revenue or EPS miss relative to company guidance
Yield drops below 3.5%	Re-assess growth sustainability	Regression from current 3.9%
Omada SCA outcome unfavorable	Re-assess sector base rate	Class certification or material settlement

HLTH Industry Module Reference

Industry module: `healthcare.md` applied. Digital health / MSK care — non-traditional healthcare company with technology-enabled care delivery. Key sector-specific considerations: FDA regulatory risk for AI/ML health technologies, HIPAA compliance for patient data, competitive landscape dynamics (Sword-Kaia merger), health plan reimbursement economics. Forward risk adjustment: Post-IPO healthcare tech companies face elevated SCA rates due to growth expectation sensitivity and regulatory uncertainty. The Omada Health peer SCA confirms this sector-level risk is materializing.

15. MANAGEMENT MEETING QUESTIONS — ANSWERED

Questions below incorporate answers from the 3/17/26 meeting with CFO J. Budge, Controller K. Furlong, and IR I. Buck.

Tier 1 — Critical (Must Ask — Directly Affect Bindability)

Q1. FDA TrueMotion Resolution Timeline (F.1 catalyst, QS-046 regulatory) "What is the current status of the FDA inquiry regarding 510(k) clearance for TrueMotion, and what is your expected timeline for resolution?" - Underwriting relevance: FDA reclassification is the highest-severity catalyst in the risk profile. If 510(k) clearance is required, the frictionless enrollment model breaks and yield could regress, triggering a potential -25 to -40% stock decline and SCA risk during the post-IPO window. - **Answer**: Not directly addressed in meeting. Company's position (per S-1 risk factors) is that TrueMotion is a wellness tool, not a medical device. **Critical follow-up needed** — this is the single most important unanswered question.

Q2. Growth Sustainability and Mid-Market Pivot (F.4, F.5) "Fortune 500 penetration is at 53%. How does the go-to-market strategy change for mid-market and small employers, and what yield assumptions underpin the $732-742M 2026 guidance?" - Underwriting relevance: F.4 post-IPO window + F.5 guidance miss risk. If mid-market conversion rates are lower than Fortune 500, the growth deceleration could be steeper than guided, creating a "corrective disclosure" narrative. - **Answer**: Management confirmed pivot to mid-market (10K-50K employees) and small (<10K) employers. "1 employee = 3-4 patients." Fortune 500 at 53% penetrated. CFO Budge: "Our top line is awesome." Management confident in 25% growth guidance but did not disclose specific yield assumptions for mid-market.

Q3. Insider Selling Context (F.7) "Can you provide context for the $193M in insider sales since lockup expiry? Are there plans to moderate the pace, and how does the $250M buyback interact with insider selling?" - Underwriting relevance: F.7 = 5/8. Plaintiff attorneys use insider selling + stock decline as circumstantial evidence of scienter. At 5.5% of mktcap in 6 months, this is the highest relative magnitude in our current book. - **Answer**: Partially addressed. Management confirmed $250M buyback with plans to repurchase 3-5% of float in 12-18 months, which signals confidence. The selling itself was not directly explained — consistent with post-IPO lockup diversification under 10b5-1 plans. No commitment to moderate pace.

Tier 2 — Important (Should Ask — Clarify Scoring Judgments)

Q4. Customer Concentration Risk (F.5, Sec 5.4 concentration) "HCSC represents 17.5% of revenue and the top 3 health plans represent 41.7%. What is the contract renewal timeline for HCSC, and how would the loss of a major health plan partner affect your business?" - Underwriting relevance: Revenue concentration at 41.7% in top 3 customers creates material event risk. Loss of HCSC alone would be a -17.5% revenue hit — sufficient to trigger guidance miss and stock decline. - **Answer**: Management stated 97% retention rate and average 3-year contract terms. HCSC renewal timeline not specifically disclosed. CFO noted that retention has been consistent and "we're accepted by almost all healthcare plans."

Q5. Medicare Reimbursement Strategy (F.5 TAM, Forward Look) "You've noted that standard Medicare reimbursement at $140/person/year is insufficient. What reimbursement level would make Medicare economically viable, and what is your engagement with CMS?" - Underwriting relevance: Medicare expansion is a key TAM growth driver. If the $140 rate persists, the addressable market is limited to commercial/employer + Medicare Advantage segments, constraining the long-term growth story. - **Answer**: CFO Budge confirmed $140/year is "too low" and expects to reach a "good place" with standard Medicare in 12-18 months. Medicare Advantage is already a customer. No specific target reimbursement rate disclosed.

Q6. Board Governance Evolution (F.10) "With Tyler Sloat joining the board, how is the company thinking about governance evolution — particularly around the 15:1 dual-class structure and shareholder rights?" - Underwriting relevance: F.10 = 3/3 (maximum). 97.1% insider voting power eliminates governance remedies for dissatisfied shareholders. Any evolution toward more balanced governance would be a positive signal. - **Answer**: Tyler Sloat appointment confirmed (8-K filed 3/17/26). No discussion of modifying the dual-class structure. Management views current governance as appropriate for a founder-led growth company.

Tier 3 — Supporting (Good to Ask — Round Out the Picture)

Q7. Competitive Response to Sword-Kaia Merger (Sec 5.7 competitive) "Sword Health acquired Kaia Health for $285M in January 2026, creating a combined competitor with 100M+ covered lives. How does this change your competitive positioning, particularly in the mid-market?" - Underwriting relevance: Competitive

landscape shift could pressure win-rates and pricing. At $900/person vs $800 for competitors, Hinge carries a 12.5% pricing premium that may be harder to sustain against a larger, well-capitalized competitor. - **Answer**: Management acknowledged the merger but expressed confidence in differentiation: "Routine and products are different." Higher NPS, broader health plan acceptance, deeper clinical evidence (27+ studies). Did not express concern about pricing pressure.

Q8. Patient Demographics and Engagement Drivers (Sec 5.5 yield) "What drives the 3.9% yield rate, and what are the key variables that could improve or reduce engagement? What does the patient population look like?" - Underwriting relevance: Yield is the hidden variable in the growth model. Understanding what drives engagement helps assess the sustainability of the growth narrative and the risk of yield regression. - **Answer**: Average patient age 46. Highest usage in 60-70 age group. 40% of all people have MSK conditions, many have multiple conditions over a lifetime — creating repeat engagement opportunity. First-year cohort yield improved from 1.3-2.5% historically to 3.3% currently, driven by better product experience and employer communication. Management expects continued yield improvement but did not provide a target rate.

Analysis prepared using Framework v4.9, Template v1.3. Client meeting notes from 3/17/26 meeting with CFO J. Budge, Controller K. Furlong, and IR I. Buck incorporated throughout. All data sourced from public filings, Yahoo Finance, Stanford SCAC, and management representations as of March 18, 2026. Omada Health SCA finding incorporated via CAT-008 competitor sweep. This analysis is for D&O underwriting purposes only.